Hypermobile Ehlers-Danlos Syndrome (hEDS)

A Patient Guide for Healthcare Providers

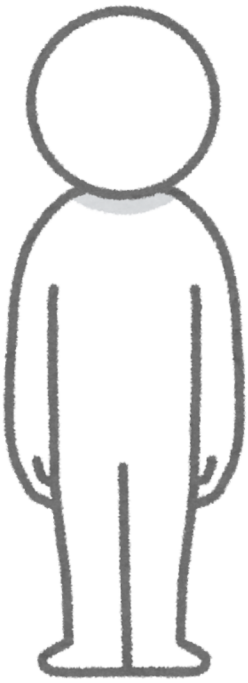
WHAT IS HEDS? hEDS is a heritable disorder of connective tissue, the structural 'glue' of the body, causing joint instability, skin fragility, and systemic effects. hEDS severity varies widely, from mild joint laxity to wheelchair use, feeding tubes, and complex multisystem involvement.

~1 in 500 people affected	Avg. 10+ years to diagnosis	3:1 to 4:1 diagnosed are female	No cure: treatment is management-focused

HOW HEDS AFFECTS THE BODY — SYSTEMIC INVOLVEMENT: Patient has checked applicable symptoms

Neurological
- ☐ Migraines & headaches
- ☐ Brain fog/cognitive fatigue
- ☐ Small fiber neuropathy
- ☐ Proprioception deficits
- ☐ Anxiety/depression

Cardiovascular
- ☐ POTS — heart rate spikes on standing
- ☐ Blood pooling & dizziness
- ☐ Palpitations



Gastrointestinal
- ☐ IBS
- ☐ Gastroparesis/delayed emptying
- ☐ GERD & acid reflux
- ☐ Food intolerances

Dermatological
- ☐ Soft, velvety, hyperextensible skin
- ☐ Stretch marks without weight change
- ☐ Easy bruising
- ☐ Poor wound healing

Immune / MCAS
- ☐ MCAS — mast cell overactivation
- ☐ Flushing, hives, itching
- ☐ GI distress & food reactions
- ☐ Chemical/environmental sensitivity

Genitourinary
- ☐ Pelvic floor dysfunction
- ☐ Bladder urgency/frequency
- ☐ Chronic pelvic pain
- ☐ Menstrual irregularities

Musculoskeletal
- ☐ Joint hypermobility & instability
- ☐ Subluxations & dislocations
- ☐ Chronic widespread pain
- ☐ Muscle fatigue & weakness
- ☐ Cervical instability (can cause neurological issues)

Fatigue & Sleep
- ☐ Profound fatigue
- ☐ Non-restorative sleep
- ☐ Post-exertional malaise
- ☐ Chronic widespread pain at rest

The hEDS Trifecta: Frequently Co-Occurring Conditions

hEDS	+	POTS	+	MCAS
Joint instability Structurally abnormal connective tissue Systemic symptoms		Heart rate spikes on standing Dizziness & fatigue Brain fog & cognitive dysfunction Exercise intolerance		Mast cell overactivation Flushing, hives, itching GI distress & food reactions Chemical/environmental sensitivity

Why Primary Care Matters for hEDS: The primary care provider is the most consequential person in an hEDS patient's medical journey — not because they are expected to be an hEDS specialist, but because they control access to the specialists who are. A referral made early changes outcomes. A referral withheld, or a case closed with a functional label, can add years to a diagnostic odyssey that already averages over a decade.

hEDS patients frequently arrive at primary care after being dismissed elsewhere. They have researched their condition out of necessity and learned to anticipate skepticism. A PCP who validates the condition, coordinates across specialties, and documents symptoms consistently is providing the structural support that makes the rest of the patient's care possible.

This condition has no cure. Management is the goal, and management requires a coordinated team.

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DO

- Treat as a multi-system condition
- Coordinate across specialties
- Validate the patient's experience
- Screen for POTS, MCAS, and GI issues
- Use low-trauma exam techniques
- Document hEDS formally in the chart to enable referrals, accommodations, and continuity across providers
- Recognize that functional capacity varies day to day — a patient who appeared well at their last visit may be significantly limited today
- Take the patient's self-knowledge seriously — Most hEDS patients have researched their condition out of necessity and are a reliable source of clinical context

DON'T

- Dismiss as "just anxiety"
- Recommend high-impact exercise
- Assume normal labs = no problem
- Overlook common comorbidities
- Over-rely on Beighton score alone
- Attribute pain reports to behavior rather than undertreated chronic illness
- Attribute cognitive symptoms to depression or anxiety before ruling out dysautonomia
- Use a patient's ability to work or appear functional as evidence their symptoms are manageable
- Close the case after a single specialist returns a normal finding — hEDS requires coordinated multisystem evaluation, not sequential elimination

ORDER / REFER

- Tilt table test or NASA Lean Test as in-office screen
- Rheumatology referral
- GI motility evaluation
- PT specialist in hypermobility
- Allergy / Immunology (MCAS)
- Mast cell mediator testing if MCAS is suspected — serum tryptase, 24-hour urine n-methylhistamine, and urine prostaglandin D2; coordinate with Allergy/Immunology
- Pain management consultation
- Genetics referral if EDS subtype is unclear or family history suggests a more complex presentation — confirmed hEDS does not require genetic testing
- Psychology referral using a trauma-informed framework — to support patients managing chronic illness, not to explain away physical symptoms

COMMON MISDIAGNOSES IN hEDS PATIENTS PRESENTING TO PRIMARY CARE

Often Diagnosed As	Consider instead/Also	Key Differentiator
Anxiety / panic disorder	hEDS with POTS and autonomic dysfunction	Positional heart rate change; symptoms worsen on standing; not purely situational
Fibromyalgia	hEDS with central sensitization	Joint hypermobility history; systemic involvement; both may coexist
Depression (primary)	hEDS with undertreated chronic pain	Mood symptoms secondary to unmanaged physical illness; treat the physical cause
Hypochondria / somatic symptom disorder	hEDS with normal labs	Clinical diagnosis expected to have normal labs; symptom pattern is physiologically consistent
Chronic fatigue syndrome / ME-CFS	hEDS with POTS and post-exertional malaise	Orthostatic assessment; POTS frequently drives fatigue in hEDS
Deconditioning	hEDS with exercise intolerance	Symptoms predate inactivity; exercise intolerance is physiological, not motivational
Growing pains (in younger patients)	Early hEDS presentation	Pain severity, systemic symptoms, and family history warrant monitoring rather than dismissal

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PLEASE COMPLETE THIS PAGE BEFORE YOUR APPOINTMENT AND BRING IT WITH YOU

MY CURRENT MEDICATIONS & SUPPLEMENTS	WHAT HELPS:
	WHAT MAKES IT WORSE:

WHAT I NEED FROM TODAY'S APPOINTMENT

Referrals needed:	
Questions I have:	
Medication changes:	
My primary concern today:	
Other:	

CURRENT SYMPTOM SEVERITY: 0-10 Refer to Mankoski Pain Scale (pg 4)

Joint pain/instability severity:
Fatigue severity:
GI symptoms type and frequency:
Heart rate/dizziness triggers and frequency:
Additional symptoms:

ADDITIONAL NOTES

Sources: Malfait et al. 2017 (AJMG) | Tinkle et al. 2017 (AJMG) | Genetics in Medicine Open 2023 | ehlers-danlos.com | PMC11390471 | PMC11348541

This document was created to provide a concise clinical reference for providers who may be less familiar with hEDS.

It is evidence-based, citation-backed, and designed to support, not replace, your clinical judgment.

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MANKOSKI PAIN SCALE

Use this scale when rating your pain severity in CURRENT SYMPTOM SEVERITY

#	What the pain is like	Typical treatment	In my own words
0	No pain.	*No medication needed.*	*"I feel completely normal."*
1	Very minor annoyance — occasional minor twinges.	*No medication needed.*	*"Hardly notice it."*
2	Minor annoyance — occasional strong twinges.	*No medication needed.*	*"Annoying but manageable."*
3	Annoying enough to be distracting.	*Mild OTC painkillers may help.*	*"Hard to ignore; affects my focus."*
4	Can be ignored if very focused, but still distracting.	*Mild OTC painkillers relieve pain for 3–4 hours.*	*"Getting in the way of tasks."*
5	Can't be ignored for more than 30 minutes.	*Mild OTC painkillers reduce pain for 3–4 hours.*	*"Stops me mid-task."*
6	Can't be ignored. Can still go to work and participate in social activities.	*Stronger prescription pain relief needed; works 3-4 hours.*	*"Present all the time. I push through."*
7	Difficult to concentrate; interferes with sleep. Can still function with effort.	*Stronger painkillers only partially effective.*	*"Hard to function. Sleep is disrupted."*
8	Physical activity severely limited. Can read/converse with effort. Nausea possible.	*Strongest painkillers minimally effective.*	*"Mostly bed-bound. May feel nauseated."*
9	Unable to speak. Crying out or moaning uncontrollably. Near delirium.	*Strongest painkillers only partially effective.*	*"Cannot communicate. Losing control."*
10	Unconscious. Pain causes passing out.	*Strongest painkillers only partially effective.*	*"Passed out or on the verge of it."*

Mankoski Pain Scale developed by Andrea Mankoski, 1995. Adapted for patient communication. Not a clinical diagnostic tool.

IMPORTANT NOTE FOR HEDS PATIENTS & PROVIDERS:

People with hEDS often have an altered pain baseline due to central sensitization — a process in which the nervous system becomes increasingly sensitized to pain signals over time.

A '3' for this patient may be what others feel as a '6.'
Please do not compare severity numbers to those of patients without chronic illness.

This scale helps us communicate:
it is not a measure of tolerance, willpower, or how 'bad' things really are.

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